



PO No :PO3794731633-544



Name	: Mr.PRADEEP GIRI	Client Name	: TATA 1MG AHMEDABAD
Age/Gender	: 26/Male	Registration Date	: 15/Dec/2024 01:04PM
Patient ID	: AMD112043	Collection Date	: 15/Dec/2024 07:42AM
Barcode ID/Order ID	: D15316774 / 11449657	Report Date	: 15/Dec/2024 07:20PM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Whole Blood-EDTA		

HAEMATOLOGY

NUTRITION CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Complete Blood Count				
Hemoglobin	15.5	g/dL	13.0-17.0	Cyanide free SLS
RBC	5.45	10 ⁶ /cu.mm	4.5 - 5.5	Impedence variation
HCT	47.4	%	40 - 50	Pulse Height Average
MCV	87.0	fL	83 - 101	Calculated
MCH	28.4	pg	27 - 32	Calculated
MCHC	32.7	g/dL	31.5 - 34.5	Calculated
RDW-CV	11.8	%	11.6-14	Calculated
Total Leucocyte Count	7.12	10 ³ /μl	4 - 10	Flowmetry DHSS/ Microscopy
Differential Leucocyte Count				
Neutrophils	38	%	40-80	DHSS/ Microscopy
Lymphocytes	42	%	20-40	DHSS/ Microscopy
Monocytes	10	%	2-10	DHSS/ Microscopy
Eosinophils	10	%	1-6	DHSS/ Microscopy
Basophils	00	%	0-2	Flowcytometry DHSS/ Microscopy
Absolute Leucocyte Count				
Absolute Neutrophil Count	2.71	10 ³ /μl	2-7	Calculated
Absolute Lymphocyte Count	2.99	10 ³ /μl	1-3	Calculated
Absolute Monocyte Count	0.71	10 ³ /μl	0.1-1	Calculated
Absolute Eosinophil Count	0.71	10 ³ /μl	0.02-0.5	Calculated
Absolute Basophil Count	0	10 ³ /μl	0.02-0.1	Calculated
Platelet Count	275	10 ³ /μl	150 - 410	Impedence Variation /Microscopy
MPV	10.7	fL	6.5 - 12	Calculated
PDW	19.9	fL	11-22	Calculated

Comment:

As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood.

DHSS : Double Hydrodynamic Sequential System



This test has been performed at
TATA 1MG AHMEDABAD
Address: Shop no. 1 to 4 , Ground floor ,
Shyam Elkin , Plot no. 38 , Near Shalin Soliter
, Shilaj Circle , Mouje Shilaj , Taluka -
Ghatlodia , Ahmedabad , 380058

Dr. Chitral Kothari
MBBS, MD (Pathology)
Consultant Pathologist
Reg No: G-22953





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Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum		

BIOCHEMISTRY

NUTRITION CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Protein Total, Serum				
Protein, Total	7.70	g/dL	5.7-8.2	Biuret
Albumin	4.70	g/dL	3.2-4.8	BCG Dye Binding
Globulin	3.0	g/dl	2.1 - 3.9	Calculated
A/G Ratio	1.57	Ratio	0.8 - 2.1	Calculated

Comment:

Protein total:

Major component of plasma; influenced by nutritional state, hepatic function, renal function, and various diseases. Serum albumin indicates severity in chronic liver disease. Useful in nutritional assessment if there is no impairment in production or increased loss of albumin. There is a 10% reduction in serum albumin level in late pregnancy (related to hemodilution)

Increased in :Dehydration, shock, hemoconcentration.

Decreased in :Decreased hepatic synthesis (chronic liver disease, malnutrition,malabsorption, malignancy).

Increased losses (nephrotic syndrome, burns, trauma, hemorrhagewith fluid replacement, fistulas, enteropathy,acute or chronic glomerulonephritis).

Hemodilution (pregnancy, CHF).

Drugs: estrogens.

Serum Electrolyte

Sodium	146	mEq/L	136-145	Indirect ISE
Potassium	5.10	mEq/L	3.5-5.1	Indirect ISE
Chloride	111.0	mEq/L	98-107	Indirect ISE

Comment:

- Kidneys actively reabsorb or excrete electrolytes to maintain the electrolyte balance of the body. Owing to their small size, almost all electrolytes are filtered at the glomerulus. After filtration, most of the electrolytes are absorbed back at the tubular level but any problem at the tubular level will result in non- absorption and excessive loss of electrolytes in urine.

NABL certificate
and scope

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Sample Type	: Serum		

BIOCHEMISTRY

NUTRITION CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Sodium along with chloride, potassium, and water is important in the regulation of osmotic pressure and water balance between intracellular and extracellular fluids.				
- An increase in sodium concentration (hypernatremia) may indicate impaired sodium excretion or dehydration. Hypernatremia is rare but occurs most often in the elderly and is often hospital-acquired. A decrease in sodium concentration (hyponatremia) may reflect over hydration, abnormal sodium loss, or decreased sodium intake, seen in conditions such as nephrotic syndrome, heart failure, generalized edema, and cirrhosis. - Hypokalemia and hyperkalemia are caused by changes in potassium intake, altered excretion, or transcellular shifts. Diuretic use and gastrointestinal losses are common causes of hypokalemia, whereas kidney disease, hyperglycemia, and medication use are common causes of hyperkalemia. When severe, potassium disorders can lead to life-threatening cardiac conduction disturbances and neuromuscular dysfunction. Therefore, a first priority is determining the need for urgent treatment through a combination of history, physical examination, laboratory, and electrocardiography findings. - Serum chloride concentration can be elevated above the normal range – hyperchloremia - either by the addition of excess chloride to the ECF compartment or by the loss of water from this compartment, and vice versa. The serum chloride concentration can be reduced below the normal range (hypochloremia) by the loss of chloride from the ECF or the addition of water to this compartment. Hyperchloremia is common in critically ill people. Severe dehydration, Diarrhea and excessive urination, Metabolic acidosis, Kidney disease, Chemotherapy may lead to hyperchloremia. Newborn babies often have hyperchloremia because their chloride levels rise in the week after birth. However, this is nothing to worry about, as the levels rise naturally and do not indicate a health problem. Hypochloremia may be seen in Low salt intake in the diet, Metabolic alkalosis, Certain medications, such as diuretics and laxatives, as these may reduce the amount of fluid in the body, Addison's Disease, etc.				

Calcium

Calcium	10.5	mg/dL	8.3-10.6	Arsenazo III
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Comment:

Increased in: Hyperparathyroidism primary and secondary, Acute and chronic renal failure, Following renal transplantation, Osteomalacia with malabsorption, Acute osteoporosis, Malignant tumours (specially of breast, lung and kidney), Drugs: Vit. D and A intoxication, Diuretics, estrogen, androgen, tamoxifen, lithium

Decreased in: Hypoparathyroidism, Surgical and Idiopathic, Pseudohypoparathyroidism, Chronic renal disease with uremia and phosphate retention, Malabsorption of Calcium and Vit.D, obstructive jaundice, Bone Disease (Osteomalacia and rickets), Drugs: Cancer chemotherapy drugs, calcitonin, loop-actives diuretics, Hypomagnesemia, Hypoalbuminemia



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Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum		

BIOCHEMISTRY

NUTRITION CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Phosphorus, Serum				
Phosphorus, Serum	3.50	mg/dL	2.4 - 5.1	Phosphomolybdate

Comment:

Phosphate metabolism is under the regulation of PTH, Vitamin D metabolites, and Fibroblast growth factor-23. Serum phosphate concentrations are about 50% higher in infants than in adults and decline throughout childhood as a consequence of the ability of growth hormone to increase the renal phosphate threshold.

Increased in:

Decreased Renal filtration (Acute or chronic renal failure) or increased reabsorption (e.g., hypoparathyroidism)
Increased Phosphate load (e.g., Oral or iv administration, Phosphate-containing laxatives or enemas, Vitamin D intoxication)
Cell Lysis (e.g., hemolysis, leukemias, chemotherapy, rhabdomyolysis)
Bone disease (e.g., healing fractures, multiple myeloma, Paget disease, osteolytic tumors)
Genetic (e.g., Hypoparathyroidism, Tumoral calcinosis)

Decreased in:

Intracellular Shift (e.g., Oral or intravenous Glucose, Insulin, Diabetic ketoacidosis, Respiratory alkalosis, Alcoholism, Severe burns)
Lowered Renal Phosphate Threshold (e.g., Primary or secondary hyperparathyroidism, Renal tubular defects)
Decreased Intestinal Absorption (Malabsorption syndrome, Vitamin D deficiency) or increased loss (Vomiting, Diarrhea)
Drugs (e.g., Salicylate, Paracetamol, Estrogens, Diuretics, Bisphosphonates, Anticonvulsants, Phosphate binding antacids, Antiviral drugs etc)

Note:

Because a significant diurnal variation in plasma phosphate has been reported, fasting morning specimens are recommended. Levels are influenced by dietary intake, meals, and exercise.

NABL certificate
and scope



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Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum		

BIOCHEMISTRY

NUTRITION CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Iron Studies, Comprehensive				
Iron Serum	97	µg/dL	65-175	Ferrozine
Total Iron Binding Capacity (TIBC)	353.4	µg/dL	240-450	Calculated
Unsaturated Iron Binding Capacity	256	µg/dL	120 - 470	Ferene
Transferrin saturation	27.45	%	16-50	Calculated
Ferritin	67.40	ng/mL	22 - 322	CLIA

Comment:

Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron is seen in iron deficiency and anaemia of chronic disorders.

Increased iron concentration are seen in hemolytic anaemias, hemochromatosis and acute liver disease. Serum Iron alone is unreliable due to considerable physiologic diurnal variation in the results with highest values in the morning and lowest values in the evening as well as variation in response to iron therapy .

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. Increased levels of TIBC suggest that total iron body stores are low, increased concentration may be the sign of Iron deficiency anaemia, polycythemia vera ,and may occur during the third trimester of pregnancy. Decreased levels may be seen in hemolytic anaemia, hemochromatosis, chronic liver disease, hypoproteinemia ,malnutrition.

Unsaturated Iron Binding Capacity (UIBC) is increased in low iron state and decreased in high iron concentration such as hemochromatosis. In case of anaemia of chronic disease the patient may be anaemic but has adequate iron reserve and a low UIBC.

Transferrin Saturation occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.

*Please note change in BRI of Ferritin.



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Patient ID	: AMD112043	Collection Date	: 15/Dec/2024 07:42AM
Barcode ID/Order ID	: D15316773 / 11449657	Report Date	: 16/Dec/2024 12:06PM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum		

BIOCHEMISTRY

NUTRITION CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Magnesium				
Magnesium	2.3	mg/dL	1.3-2.7	Xylidyl blue

Comment:

Magnesium (Mg) is an important cation essential for the function of more than 300 cellular enzymes.Total body Mg depends on GI absorption and renal excretion.50 - 60% of body magnesium content is stored in the bones.

Increased levels (Hypermagnesemia) : Acute & chronic renal failure, Addison's disease, Diuretics, antacids & laxative use, Hypothyroidism, Elderly diabetics.

Decreased levels (Hypomagnesemia) : Chronic nephritis, Acute pancreatitis, Alcoholic cirrhosis.



This test has been performed at
TATA 1MG MUMBAI
Address: 1st floor, A Wing, Krislon House,
Saki Vihar Rd, opp. Ansa Industrial Estate,
behind Picnic Hotel, Saki Naka, Mumbai,
Maharashtra 400072

G. G. Hukkerikar

Dr Gaurang Hukkerikar
MBBS, MD (Pathology)
Consultant Pathologist
Reg No: 2013/05/1766



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Patient ID	: AMD112043	Collection Date	: 15/Dec/2024 07:42AM
Barcode ID/Order ID	: D15316773 / 11449657	Report Date	: 17/Dec/2024 04:27PM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum		

BIOCHEMISTRY

NUTRITION CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Zinc, Serum				
Zinc,Serum	176.00	µg/dL	46-150	Enzymatic

Result rechecked. Kindly correlate clinically.

Comment:

Zinc is an essential element; it is a critical cofactor for carbonic anhydrase, alkaline phosphatase, RNA and DNA polymerases, alcohol dehydrogenase, and many other physiologically important proteins. Zinc is a key element required for active wound healing. Zinc depletion occurs either because it is not absorbed from the diet (excess copper or iron interfere with absorption) or it is lost after absorption. Dietary deficiency may be due to absence (parenteral nutrition) or because the zinc in the diet is bound to phytate (fiber) and not available for absorption.

Decreased Levels: Treatment with anabolic steroids & metal chelating drugs, synthetic diet therapies, Acrodermatitis enteropathica, Alopecia, Typhoid, Pulmonary tuberculosis, liver metastasis, Celiac sprue, Thalassemia major, Pernicious anemia, Acute Myocardial infarction, Renal disease, pregnancy, lactation & old age.

Increased levels: Hemodialysis with zinc containing dialysate, Primary osteosarcoma of bone, Coronary heart disease, Arteriosclerosis, Familial hyperzincemia & Anemia.

NABL certificate
and scope

This test has been performed at

TATA 1MG OKHLA

Address: 2nd Floor, B-225, Okhla Phase I,
Okhla Industrial Estate, New Delhi, Delhi
110020

Dr. Dhananjay Singh
MBBS, MD(Pathology)
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Barcode ID/Order ID	: D15316773 / 11449657	Report Date	: 15/Dec/2024 07:08PM
Referred By	: Dr.	Report Status	: Final Report
Sample Type	: Serum		

IMMUNOLOGY

NUTRITION CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Vitamin D (25-OH)				
Vitamin D (25-OH)	7.4	ng/mL	Deficiency:< 20, Insufficiency:20-29, Sufficiency:30 - 100, Toxicity possible:> 100	CLIA

Comment:

- Vitamin D is a fat-soluble steroid prohormone involved in the intestinal absorption of calcium and the regulation of calcium homeostasis.
- Two forms of vitamin D are biologically relevant - vitamin D3 (Cholecalciferol) and vitamin D2 (Ergocalciferol).
- Both vitamins D3 and D2 can be absorbed from food but only an estimated 10-20perc. of vitamin D is supplied through nutritional intake.
- Vitamin D is converted to the active hormone 1,25-(OH)2-vitamin D (Calcitriol) through two hydroxylation reactions. The first hydroxylation converts vitamin D into 25-OH vitamin D and occurs in the liver. The second hydroxylation converts 25-OH vitamin D into the biologically active 1,25-(OH)2-vitamin D and occurs in the kidneys as well as in many other cells of the body.
- Most cells express the vitamin D receptor and about 3perc. of the human genome is directly or indirectly regulated by the vitamin D endocrine system.
- The major storage form of vitamin D is 25-OH vitamin D and is present in the blood at up to 1,000 fold higher concentration compared to the active 1,25-(OH)2-vitamin D. 25-OH vitamin D has a half-life of 2-3 weeks vs. 4 hours for 1,25-(OH)2-vitamin D. Therefore, 25-OH vitamin D is the analyte of choice for determination of the vitamin D status.
- Risk factors for vitamin D deficiency include low sun exposure, inadequate intake, decreased absorption, abnormal metabolism, vitamin D resistance and liver or kidney diseases.
- Vitamin D deficiency is a cause of secondary hyperparathyroidism and diseases resulting in impaired bone metabolism (like rickets, osteomalacia).
- Recently, many chronic diseases such as cancer, high blood pressure, osteoporosis and several autoimmune diseases have been linked to vitamin D deficiency.
- The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D

Utility Quantitative determination of 25-hydroxyvitamin D (25-OH vitamin D).



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IMMUNOLOGY

NUTRITION CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Vitamin B12				
Vitamin B12	242.0	pg/ml	211 - 911	CLIA

Comment:

- Vitamin B12** along with **folate** is essential for DNA synthesis and myelin formation.
- Decreased levels** are seen in anaemia, term pregnancy, vegetarian diet, intrinsic factor deficiency, partial gastrectomy/ileal damage, celiac disease, oral contraceptive use, parasitic infestation, pancreatic deficiency, treated epilepsy, smoking, hemodialysis and advanced age.
- Increased levels** are seen in renal failure, hepatocellular disorders, myeloproliferative disorders and at times with excess supplementation of vitamins pills.

Vitamin B9 (Folic Acid)

Vitamin B9 (Folic Acid)	3.21	ng/mL	0.35-3.37 Deficient 3.38-5.38 Indeterminate >5.38 Normal	CLIA
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Comment:

Folate plays an important role in the synthesis of purine & pyrimidines in the body and is important for the maturation of erythrocytes. It is widely available from plants and to a lesser extent organ meats, but more than half the folate content of food is lost during cooking. Folate deficiency is commonly prevalent in alcoholic liver disease, pregnancy, and the elderly. It may result from poor intestinal absorption, nutrition deficiency, excessive demand as in pregnancy or in malignancy, and in response to certain drugs like Methotrexate & anticonvulsants. It is now routine practice to recommend dietary folate supplements from conception to the 12th week of pregnancy; such supplementation has been proven to reduce the incidence of neural tube defects.

Decreased Levels: Megaloblastic anemia, Infantile hyperthyroidism, Alcoholism, Malnutrition, Scurvy, Liver disease, B12 deficiency, dietary amino acid excess, adult Celiac disease, Tropical Sprue, Crohn's disease, Hemolytic anemias, Carcinomas, Myelofibrosis, vitamin B6 deficiency, pregnancy, Whipple's disease, extensive intestinal resection, and severe exfoliative dermatitis.

Note:

Certain drugs like Pyrimethamine, methotrexate, and trimethoprim are all folate antagonists i.e. they stop the action of the folic acid; phenytoin can decrease the intestinal absorption of folates, and ethanol both decreases absorption and increases excretion of folic acid.



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IMMUNOLOGY

NUTRITION CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval	Method
To differentiate vitamin B12 & folate deficiency, measurement of Methylmalonic acid in urine & serum Homocysteine level is suggested.				

*** End Of Report ***

Conditions of Laboratory Testing & Reporting:

Test results released pertain to the sample, as received. Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the interpreting clinician. Result delays may happen because of unforeseen or uncontrollable circumstances. Test report may vary depending on the assay method used. Test results may show inter-laboratory variations. Test results are not valid for medico-legal purposes. Please mail your queries related to test results to Customer Care mail ID care@1mg.com

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NABL certificate and scope



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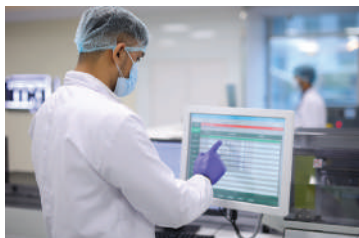
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